

Daily Content Intel

August 4, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, so there's a new systematic review out of the University of Kansas, 48 studies on noncontact ACL injuries, and they went looking somewhere the knee research usually skips. The foot.

Here's what they found. Athletes who landed on their forefoot were protected. Athletes who landed flat footed or heel first, or with their toes turned in or turned out, kept showing up in the higher risk group. Same story with medial arch collapse, that navicular drop where the arch flattens under load, and with greater side to side rearfoot motion.

Prior ankle injury was in there too, which tracks with what I see. An athlete rolls an ankle in September, it gets iced for a week, they're back at practice, and now they're cutting off a foot that can't organize itself, and the knee ends up eating that.

So what do you do Monday? Watch the foot on landings. Film a drop jump from the front and look at two things, whether the arch dumps and where the toes point when they hit. If the arch collapses or the foot spins, that's your first target, and it's a lot easier to train than a reconstructed knee.

The review is honest that the study quality here is mostly low, so this is a place to look, probably not a rule yet.

Be Good. Help Someone. Learn Lots.

Hook: Your ACL tear might start at your foot.

Spoken script (word for word):

Okay, so there's a new systematic review out of the University of Kansas, 48 studies on noncontact ACL injuries, and they went looking somewhere the knee research usually skips. The foot.

Here's what they found. Athletes who landed on their forefoot were protected. Athletes who landed flat footed or heel first, or with their toes turned in or turned out, kept showing up in the higher risk group. Same story with medial arch collapse, that navicular drop where the arch flattens under load, and with greater side to side rearfoot motion.

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that's your first target, and it's a lot easier to train than a reconstructed knee.

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On-screen text seeds:

- 0:00 Your ACL tear might start at your foot
- 0:07 48 studies. Noncontact ACL injuries.
- 0:18 Forefoot landing = protective
- 0:26 Flat foot, heel first, toe-in, toe-out = higher risk
- 0:34 Arch collapse (navicular drop)
- 0:45 That "minor" ankle roll in September
- 0:58 Monday: film a drop jump from the FRONT
- 1:08 Watch the arch. Watch the toes.
- 1:20 Easier to train than a reconstructed knee

Caption (copy-paste ready):

Your ACL tear might start at your foot.

A new systematic review pulled 48 studies on noncontact ACL injuries and looked at the joint below the knee. Forefoot landings were protective. Flat and heel-first landings, toes turned in or out, arch collapse, and extra side-to-side rearfoot motion all clustered in the higher-risk group. Prior ankle injury showed up too.

That last one is the part I see most. An ankle gets rolled early in the season, it gets iced, and the athlete goes back to cutting off a foot that never got rebuilt. The knee pays for it later.

Monday version: film a drop jump from the front. Watch whether the arch dumps and where the toes point on contact. Train what you find.

Study quality here is mostly low, so treat this as a place to look rather than a rule.

It's time to cross your threshold.

Dr. Lars Stevenson, PT, DPT
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#aclprevention #acl #footballtraining
#lacrosse #athletictraining #sportspt

#physicaltherapy #returntosport
#youthsports #strengthandconditioning
#injuryprevention #highschoolfootball
#kneepain #sportsmedicine

Personalize this

- Which of your HS football or lacrosse athletes came to you with a knee complaint where the real finding was at the ankle or foot? Name the pattern you saw on their landing (arch dump, toe-out, stiff ankle) and what you changed first.
- You've made return-to-sport calls on ACL athletes. Have you ever held someone back specifically because their foot and ankle weren't ready, even when the knee tested fine? That story is the whole Reel.
- Where does this land in CROSS, and what does the in-season version look like for a football athlete who rolls an ankle in Week 3 and refuses to miss a game?

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC13389134/> | "Foot and Ankle Contributions to Noncontact ACL Injury

Risk: A Systematic Review"

- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC13389134/> | "Greater dynamic rearfoot motion (eversion/inversion), increased dorsiflexion, and medial arch collapse (eg, navicular drop) were commonly associated with increased risk."
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC13389134/> | "Forefoot landings were protective, while rearfoot and flatfoot strikes, as well as toe-in or toe-out positions, were consistently linked to higher injury risk."

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC13389134/>
- <https://pubmed.ncbi.nlm.nih.gov/42488193/>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, hot take. If your athlete is drinking a beet shot before a game to run faster, save the money.

New study in the Journal of the International Society of Sports Nutrition. 12 trained male sprinters, randomized, double blind, placebo controlled. They got 70 milliliters of beetroot juice or a placebo, then they jumped, and then they ran a 60 and a 100.

The juice absolutely worked at the chemistry level. Salivary nitrate went up more than 20 fold. So it got in, it was absorbed, the dose was real.

And then nothing moved.

Countermovement jump, same. Squat jump, same. 60 meter, same. 100 meter, same. They even measured contractile properties of the quad, the

hamstring, and both calves with tensiomyography, and found basically nothing there either.

Here's my read. Nitrate has decent evidence in longer efforts where oxygen cost is the limiter. A 100 meter is over before that mechanism ever becomes the thing holding you back. So the athlete drinking beets to get faster is buying a solution to a problem they don't have.

It's 12 guys and one acute dose, so hold it loosely. But if you want to get faster, sprint at full speed every week, sleep 9 hours, and get strong. That's the stack, and it's free.

Be Good. Help Someone. Learn Lots.

Hook: The beet shot before your game is doing nothing for your speed.

Spoken script (word for word):

Okay, hot take. If your athlete is drinking a beet shot before a game to run faster, save the money.

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The juice absolutely worked at the chemistry level. Salivary nitrate went up more than 20 fold. So it got in, it was absorbed, the dose was real.

And then nothing moved. Countermovement jump, same. Squat jump, same. 60 meter, same. 100 meter, same. They even measured contractile properties of the quad, the hamstring, and both calves with tensiomyography, and found basically nothing there either.

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faster is buying a solution to a problem they don't have.

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On-screen text seeds:

- 0:00 The beet shot is doing nothing for your speed
- 0:08 12 trained sprinters. Randomized. Double blind.
- 0:16 70 mL beetroot juice vs placebo
- 0:26 Salivary nitrate: up 20x. The dose worked.
- 0:35 CMJ: same
- 0:38 Squat jump: same
- 0:41 60m: same
- 0:44 100m: same
- 0:55 Nitrate helps where oxygen cost is the limiter
- 1:02 A 100m is over before that matters

- 1:14 Free stack: sprint, sleep 9 hours, get strong

Caption (copy-paste ready):

The beet shot before the game is doing nothing for your speed.

12 trained male sprinters, randomized, double blind, placebo controlled. 70 mL of beetroot juice or placebo, then jumps and a 60 and a 100.

The dose worked. Salivary nitrate went up more than 20 fold, so it was absorbed. Then countermovement jump, squat jump, 60 meter and 100 meter times all came back the same as placebo. Tensiomyography on the quad, hamstring and both calves showed nothing either.

My read: nitrate has real evidence in longer efforts where oxygen cost is the limiter, and a 100 meter finishes before that mechanism ever becomes the ceiling. An athlete buying beets for speed is solving a problem they don't have.

12 subjects and one acute dose, so hold it loosely. But the speed stack that actually

works is free. Sprint at full speed every week. Sleep 9 hours. Get strong.

It's time to cross your threshold.

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#sportsnutrition #beetrootjuice #sprintspeed
#speedtraining #footballtraining #lacrosse
#strengthandconditioning #youthsports
#athleticperformance #supplements
#sportsscience #highschoolfootball
#trackandfield #sportspt

Personalize this

- Which supplement do you get asked about most by the parents of your football and lacrosse athletes, and what's your standard answer? If beets aren't the top ask, swap the product in the hook to whatever actually is.
- Have you had an athlete whose parents were spending real money on supplements while the kid was sleeping 6 hours and never touching true max velocity work? Tell that specific contrast instead of the generic version.

- What is your actual weekly speed dose for a HS football athlete in the offseason (how many max velocity exposures, what distances)? Giving the real prescription is what separates this from every other "supplements don't work" Reel.

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC13185055/> | "Acute beetroot juice ingestion fails to improve sprint performance and neuromuscular function in trained male sprinters"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC13185055/> | "Beetroot juice ingestion did not affect countermovement jump height"
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC13185055/> | "did not enhance sprint performance or the neuromuscular function of male trained sprinters"

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC13185055/>

- <https://pubmed.ncbi.nlm.nih.gov/4214890>
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Reference

Runner-up topics

- **Resisted sprint, heavy vs light sled** (Sports Health 2026, PMID 41351337): athletes new to sled work improved more at 80% body mass than 20%, 2x/week for 6 weeks. Counters the standard "start light and progress" coaching. Held only because the sled family ran 2026-06-16 and 2026-07-01.
- **Cold water immersion is goal-dependent** (BMC 2026, 87 RCTs, 2,313 participants): passive recovery beat CWI for resistance-training strength, but cold and long helped team-sport power. A sharper take than the usual "ice baths kill gains." Held because cold plunge ran 2026-06-29 with two existing inbox notes.
- **Neuromuscular training warm-ups in youth soccer** (Front Sports Act Living 2026, 5 RCTs, n=6,447): ankle injury reduction was the one signal that held; overall and knee prevention were inconclusive and heterogeneous. Good "what the warm-up actually buys you"

piece.

Sources scanned today

- **Newsletter digest**
(newsletter_digest_2026-08-04.pdf): 2 sources, both Dr. Mercola promotional sends (gut microbiome teaser, "diagnosis that left millions searching"). Zero educational links, no extractable finding. Skipped.
- **PubMed recency sweeps** (2026[dp], sorted by date): ankle sprain prevention, ACL/hamstring/ankle prevention in HS athletes, youth strength training RCTs. Surfaced the Kansas foot-and-ankle/ACL systematic review (Orthop J Sports Med, 2026 Jul 21). That became Draft A.
- **PubMed / JISSN**: acute beetroot juice in trained sprinters (2026 May): null result, became Draft B.
- **Sports Health 2026**: "First Time Doing Resisted Sprint Training?" (80% BM vs 20% BM sled, PMID 41351337). Strong paper, held as runner-up because the sled/resisted-sprint family already ran

2026-06-16 and 2026-07-01.

- **BMC Sports Sci Med Rehabil 2026** CWI network meta-analysis (87 RCTs, 2,313 participants, PMID 41845491). Held as runner-up because cold plunge ran 2026-06-29 and has two Research-Inbox notes already.
- **J Clin Med**: "A Systematic Review of Rehabilitation Interventions for Athletes with Chronic Ankle Instability": rejected, findings too boilerplate for a screenshot target ("an improvement in balance was observed").
- **Front Sports Act Living 2026**: neuromuscular training in youth soccer meta-analysis (5 RCTs, n=6,447). Runner-up.
- **Blocked / unreachable**: journals.sagepub.com returned HTTP 403; link.springer.com forced an auth redirect. Both were recovered through their PubMed/PMC mirrors.